EL NIÑO ACTION BRIEFING · 30 OCTOBER 2026

Agadez, Agadez Region

The winter looks quiet on the heat tracker, but it is not: meningitis from January, a failed pasture, and the desert roads are where people die here, so antibiotics must be at every health post before 15 December.

REGION	EXCESS HEAT DEATHS	WINDOW	DOMINANT HAZARD
Agadez Region (GADM NER.1_1), region level	93 forecast Oct 2026 – Mar 2027, 43 of them in March; 310 already past (Jun–Sep)	6-month forecast, Oct 2026 – Mar 2027	Meningitis Jan–May; heat from March; pastoral drought; desert travel; the 2027 lean season

24/24 recommendations checked. 3 confirmed, 17 corrected, 3 unverified, 1 contraindicated. That covers the 8 ranked actions (all corrected), the 7 "Do not do this" cards (3 confirmed, 2 corrected, 1 unverified, and 1 contraindicated: a draft resident step promising herders drought cash), and the 9 For you steps (7 corrected, 2 kept as labelled general safety advice). Exposure figure: Climate Impact Lab El Niño Heat Mortality Tracker data, version of 26 October 2026, pre-peer-review. It is published for Agadez Region, not for the town, and counts heat only. No town share was calculated, because the town's population is contested. The funding target moved from the panel's XOF 45,503,350 (US$77,500) to XOF 51,374,750 (US$87,500) after verification (+13%). The Climate Impact Lab Adaptation Inventory loads in the browser and returned no entries to the research agents, so no precedent could be confirmed there.

EXECUTIVE SUMMARY — AT A GLANCE



16.97°N 7.99°E, Niger · Natural Earth

- **The risk.** A "very marked" pasture deficit, meningitis from January, and 43 forecast heat deaths in March; here the worst deaths come in the lean year after.
- **For you.** By 31 December, check that everyone aged 1–19 had the free 2025 meningitis vaccine; if not, ask at the CSI.
- **The money.** XOF 22.0–168.8 million (US$37,500–287,500); meningitis treatment stock at the CSIs buys the most lives.
- **Do not.** Do not tell herders drought-protocol cash is coming: it covers 500 households in all of Niger.

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* Suggested formulation: check with a health worker before sharing.

FOR YOU, AS A RESIDENT 9 STEPS

For a family in Agadez, this El Niño arrives in stages: meningitis from January to May, cold nights in December to February, heat from March, and a hungry 2027 lean season after a very poor pasture year. The deadliest single event of 2026 happened on the desert road. The ranked actions below are written for the region and its partners. These are the ones you can take yourself.

From January to May, take anyone with a fever and a stiff neck to the CSI the same day, and a baby who is floppy or has a bulging soft spot

PARTIALLY CONFIRMED FROM ACTION 1

BY WHEN	COST TO YOU	PROTECTS
January–May 2027	free for under-fives; for others the drugs are usually donated in epidemics, so ask, and do not delay	children and young adults

In 2024 Agadez had Niger's second-highest meningitis attack rate, 11.5 cases per 100,000, and 6.1% of cases nationally died. Early antibiotics are what keep that number down. Niger's free-care policy covers all care for children under five. It does not guarantee free meningitis treatment for older people, and health facilities are owed more than XOF 50 billion (US$85 million) in free-care arrears, so a CSI may charge.*

Check that everyone in the household aged 1–19 got the meningitis vaccine in November–December 2025; if anyone missed it, or a child has turned 1 since, ask at the CSI

PARTIALLY CONFIRMED FROM ACTION 1

BY WHEN	COST TO YOU	PROTECTS
31 December 2026	none; the vaccine was free	everyone aged 1–19

Niger ran its first preventive campaign with the new five-strain vaccine (Men5CV) in November–December 2025, for ages 1–19, across the whole country, and reached 12.3 million people (WHO, September 2026). No source names Agadez, but the government described it as nationwide. Children born since are not covered until routine vaccination at 16 months starts, and that start could not be confirmed.

* Suggested formulation: check with a health worker before sharing.

On cold nights, never burn a charcoal brazier (kanoun) in a closed room; leave a window or door open, or put it out before you sleep

UNVERIFIED FOR NIGER · ROLE-SPECIFIC

BY WHEN	COST TO YOU	PROTECTS
December 2026 – February 2027	none	everyone sleeping in the room

General safety advice, not an Agadez finding. January nights in Agadez average about 13.5°C, with rare lows near 2°C. A brazier in a closed room gives off carbon monoxide, which kills without warning: two people died this way in Buea, Cameroon, in August 2026, and France records such deaths every year. No case series for Niger was found.*

Before 1 March, keep two clean, covered 20-litre jerrycans of drinking water, and refill them every week

UNVERIFIED · ROLE-SPECIFIC

BY WHEN	COST TO YOU	PROTECTS
1 March 2027	about XOF 3,000–5,000 (US$5–9) for two jerrycans, an unsourced estimate	drinking water in the hottest weeks

A sensible precaution with no evidence base behind it. April and May in Agadez reach about 40–43°C, and the region hit 48°C in April 2024. A water store means nobody has to go without in a supply break or on the hottest days. No published market price for jerrycans was found.

When the heat returns in March: drink, wet your skin and clothing, stay in shade, and rest at midday; do not rely on a fan for an older person, and look in on elderly neighbours living alone

PARTIALLY CONFIRMED · FROM ACTION 3

BY WHEN	COST TO YOU	PROTECTS
March–May 2027	none	older people, people with high blood pressure, and young children

* Suggested formulation: check with a health worker before sharing.

Of the 93 heat deaths forecast for the region from October to March, 43 fall in March, and April and May are hotter still. In Agadez's very dry heat, a fan on dry skin can heat the body: a 2024 review advises against fans above 35°C for people over 65, and modelling puts the ceiling at 37–38°C for older adults. Wet skin cools; moving air on dry skin does not. Elderly people alone in old-town banco houses are the ones nobody sees.*

Get oral rehydration salts (ORS) sachets at the CSI; if there are none, mix 6 level teaspoons of sugar and ½ level teaspoon of salt in 1 litre of clean water, and throw it away after 24 hours

PARTIALLY CONFIRMED · FROM ACTION 3

BY WHEN	COST TO YOU	PROTECTS
before 1 March 2027	well under XOF 100 (US$0.17) per litre for the home mix, an estimate	a child with diarrhoea; anyone dehydrated by heat

This is the WHO-quoted home recipe for when sachets are not available. An older recipe (8 spoons of sugar and 1 of salt) still circulates. Too much salt is the main danger, which is why sachets come first.*

Buy grain from now through December, in 25 kg lots if you cannot afford a whole sack, and check the weight

PARTIALLY CONFIRMED · FROM ACTION 6

BY WHEN	COST TO YOU	PROTECTS
November–December 2026	under XOF 29,500 (US$50) for 100 kg of millet at June's lean-season price of XOF 295/kg; 25 kg at that price is about XOF 7,400 (US$12.60)	food for the 2027 lean season

In June 2026, the government market service (SIMA) put millet in Agadez commune at XOF 295/kg, above the national 271. Prices should be lower after the harvest, but no current Agadez figure was found. Lean-season prices rise from May, by up to 25% for millet. The national statistics office warns that sacks often weigh less than declared. A whole sack is several weeks' income, so 25 kg at a time beats going into debt.

* Suggested formulation: check with a health worker before sharing.

If you keep animals: sell the weak ones in November–January, and ask the Croix-Rouge Nigérienne in Agadez and your chief whether a beneficiary list is being drawn up after the November forage results; do not count on the money

PARTIALLY CONFIRMED · FROM ACTION 4

BY WHEN	COST TO YOU	PROTECTS
November 2026 – January 2027	none; selling early earns more	the herd's value and the household's food

The regional pastoral service (CILSS/AGRHYMET) rates Agadez's 2026 fodder deficit "très marqué". In 2010 herders who waited sold into a falling market while grain rose. In June 2026 a goat still bought about one unit of millet (terms of trade 0.99). If a list is drawn up, make sure vulnerable herding households, especially those headed by women, are on it.

Before any desert trip from March, carry at least 10 litres of water per person for each day planned, plus two days spare; leave your route and arrival time with someone in town, and travel with a second vehicle where you can

PARTIALLY CONFIRMED · FROM ACTION 2

BY WHEN	COST TO YOU	PROTECTS
every trip, from March 2027	the water and containers	everyone in the vehicle

On 4 June 2026, 49 people died of thirst about 80 km west of Assamaka after their truck broke down. They were Nigerien gold-site workers going home from Mali for Eid. Two survivors walked out for help, and the patrol that came then found a second truck stuck for three days and saved its 60 passengers. In heat above 40°C, 10 litres is only one to two days of survival.*

What to ask of the people in charge

- Ask the Direction régionale de la santé publique (DRSP): "Is ceftriaxone in stock at our CSI by 15 December?" (action 1).
- Ask the governor and the Protection Civile: "What is the one number to call when a vehicle is stranded in the desert?" (action 2).
- Ask the DRSP, the weather service (DMN), and the radio stations: "Will heat warnings go out in Hausa and Tamasheq by 1 March?" (action 3).

* Suggested formulation: check with a health worker before sharing.

WHAT THIS PLACE SHOULD DO RANKED

Ranked by expected lives saved per unit of effort before the peak. Backing lenses are shown per action; an action several lenses reached independently is the stronger bet.

1. Meningitis: by 15 December, stock treatment and rapid tests at every health post, check last year's vaccine coverage and catch up the children missed, and have the emergency vaccine request ready to send

PARTIALLY CONFIRMED | EMERGENCY MANAGER · PUBLIC HEALTH · HISTORIAN · PHILANTHROPIST · SKEPTIC | ADAPTATION INVENTORY: NOT QUERYABLE

OWNER	BY WHEN	COST	PREVENTS
DRSP Agadez, with the CHR and WHO/UNICEF	15 December 2026; weekly surveillance from then to May	XOF 3,000,000–5,871,400 (US$5,110–10,000), in kind via WHO/UNICEF	meningitis deaths from late treatment

The DRSP stocks ceftriaxone and rapid tests at the regional hospital (CHR), every CSI in the commune, and Assamaka, and runs weekly surveillance by health area. It checks Men5CV coverage from the November–December 2025 campaign by health area and gives catch-up doses to missed 1–19-year-olds through routine services. The request to the International Coordinating Group (ICG) for outbreak vaccine is pre-drafted, so it can go within 48 hours of the epidemic threshold being crossed. In Katsina, Nigeria, in 1996, treatment cost about XOF 232,500 (US$396) per death averted, against about XOF 3.5 million (US$6,000) for vaccination started six weeks late (Veeken, Ritmeijer, and Hausman, Bull WHO 1998). A simulation on Niger data found surveillance by health area worked best, and acting two weeks earlier doubled efficiency (Maïnassara et al. 2015). In 2024 Agadez had the second-highest attack rate in Niger, 11.5 per 100,000; nationally 123 of 2,012 cases died (6.1%).

* Suggested formulation: check with a health worker before sharing.

Corrected in verification: no new mass campaign. Niger already ran the first Gavi-supported preventive Men5CV campaign, with typhoid vaccine, for ages 1–19 nationwide in November–December 2025 (12.3 million people, WHO WER 101(38), 25 September 2026); Agadez is likely covered but not named. The Katsina figure is in 1996 dollars, for serogroup A and oily chloramphenicol, so use it as a ratio, not a price. The simulation used Tahoua, Tillabéri, and Dosso, not Agadez. Diphtheria is also active: 242 of Niger's 949 cases in 2026 to week 38 were in Agadez (a secondary report citing WHO AFRO, not checked at source).

2. Agree one desert-distress alert number by 1 December and keep a fuelled standby vehicle at Assamaka and Arlit from 1 March, for everyone who travels the desert; give expelled people basic relief without conditions

PARTIALLY CONFIRMED | EMERGENCY MANAGER · FRONTLINE · SKEPTIC · PUBLIC HEALTH · PHILANTHROPIST | ADAPTATION INVENTORY: NOT QUERYABLE

OWNER	BY WHEN	COST	PREVENTS
Governor of Agadez, with the Protection Civile, Alarme Phone Sahara, and IOM	alert number 1 December 2026; vehicle on standby from 1 March 2027	diesel about XOF 700,000 (US$1,192); 2,000 blankets and mats about XOF 10 million (US$17,032); both estimates	deaths of thirst after a breakdown, while nobody knows a vehicle is missing

The 49 deaths of 4 June 2026 came after a breakdown that nobody knew about until two survivors walked out; the same patrol then saved 60 people from a second truck. One agreed number and a vehicle ready to go shortens that delay. Expulsions from Algeria into Niger reached a record 34,236 in 2025 (Alarme Phone Sahara), after 31,404 in 2024, while IOM's budget fell about 30% in 2025. From December to February, blankets and mats for expelled people arriving with nothing are basic relief.

* Suggested formulation: check with a health worker before sharing.

Corrected in verification: the June 2026 dead were Nigerien gold-site workers returning from Harouba, Mali, not expelled migrants, so the vehicle serves every desert traveller. No hypothermia death among expelled people is documented (January mean minimum about 13.5°C; the record low is disputed), so the blankets are relief, not a proven life-saver. Médecins du Monde worked in Agadez from 2014 through the 2025 surge and MSF is documented at Assamaka, but neither's 2026 status could be confirmed: check authorisation before naming either. The vehicle beyond the diesel estimate is unfunded.

3. Put a heat plan in place by 1 March that works without grid power: a forecast trigger, ORS, a backed-up cooled triage room, outreach to those most at risk, and radio messages in Hausa and Tamasheq

PARTIALLY CONFIRMED | PUBLIC HEALTH · EMERGENCY MANAGER · PHILANTHROPIST · FRONTLINE · SKEPTIC | ADAPTATION INVENTORY: NOT QUERYABLE

OWNER	BY WHEN	COST	PREVENTS
DRSP Agadez and the CHR, with the Direction de la Météorologie Nationale (DMN)	1 March 2027	XOF 1,500,000–8,807,100 (US$2,555–15,000); no Sahel costing precedent	the March–May heat deaths, 43 forecast in March alone

A trigger on DMN maximum-temperature forecasts; ORS stock and a cooled triage room with solar or diesel backup at the CHR; CSI outreach to people with high blood pressure, households with under-fives, and the transit centres. The advice is to drink, wet skin and clothing, stay in shade, and rest at midday. Messages go out on state radio (ORTN, La Voix du Sahel) and community stations in Hausa, Tamasheq, and French, and through the imams and the sultanate. In 2007 about 36% of adults in Niger had high blood pressure (STEPS survey), and in Niamey screening only 3.4% of those found were being treated. Ahmedabad's heat action plan avoided an estimated 1,190 deaths a year (95% CI 162–2,218; Hess et al. 2018).*

* Suggested formulation: check with a health worker before sharing.

Transferability: weak. Ahmedabad is a humid Indian megacity, and its effect on days above 40°C (rate ratio 0.95) was not significant. No operational heat-health warning system exists in the Sahel (ACASIS was a pilot in Senegal and Burkina Faso; a 2026 ECMWF proposal would pilot one with AGRHYMET), so Agadez would be a first. The blood-pressure figures are from Niamey. Three journalists of the local station Sahara FM were arrested in May 2025, and no sign it broadcasts in 2026 was found: check before naming it. Release the upper figure only against a DRSP line-item budget.

4. Herders: act on the November forage and Cadre Harmonisé results: destock early and pre-position feed at the January pre-alert, get vulnerable households on any list, and push for Agadez in the 2027 Plan de Soutien

PARTIALLY CONFIRMED | ECONOMIST · HISTORIAN · FOOD · PHILANTHROPIST · EMERGENCY MANAGER | ADAPTATION INVENTORY: NOT QUERYABLE

OWNER	BY WHEN	COST	PREVENTS
Regional Livestock Directorate and herder associations, with the Croix-Rouge Nigérienne (CRN) Agadez and quartier and village chiefs	**decide in December 2026; act at the January pre-alert**	**pastoral cash XOF 117,428,000 (US$200,000), released on the November results; feed not costed**	**herd collapse and the 2027 lean-season hunger that follows it**

CILSS rates Agadez's 2026 pastoral deficit "très marqué", and FEWS NET links the poor season to El Niño. In Niger in 2010, the ICRC, Oxfam, and VSF-B bought about 36,000 head at pre-crisis prices, but a lessons workshop found action should have started at the January pre-alert with feed pre-positioned; feed requested was 10,000 t against 16,000 t needed. The philanthropist's pastoral cash (1,000 households at XOF 117,428 each) is a real gap. Release it on the November Cadre Harmonisé and forage results, through CRN Agadez.

Corrected in verification, and the corrections are large. The Red Cross drought early-action protocol (IFRC EAP2024NE04, valid five years from May 2024) totals CHF 874,573 (about XOF 612 million), of which CHF 607,309 (about XOF 425 million) is early action. Its November forage trigger pays XOF 40,000 × 3 months × 500 households, XOF 60 million (US$102,000), for all of Niger; it names no regions, and no 2025–26 activation was found. The UN CERF drought framework's only activation was in 2022 (XOF 5.6 billion, US$9.5 million, south-west Niger); Agadez coverage is unverified, so ask OCHA but expect no money. No African Risk Capacity policy for Niger was found for 2025–27.

* Suggested formulation: check with a health worker before sharing.

The Plan de Soutien is built from the November Cadre Harmonisé and published about February; the 2022 plan listed Agadez communes, but no 2026–27 plan was found. In 2022 the CRN reached about 6,000 of 22,000 targeted households nationally, so its capacity to absorb this is a risk. The transfer size is an assumption, and feed is uncosted because no Agadez feed price exists.

5. Count the dead from November: a daily burial tally at the cemeteries and a CHR death log, with verbal autopsy from March

PARTIALLY CONFIRMED | SKEPTIC · PUBLIC HEALTH · HISTORIAN | ADAPTATION INVENTORY: NOT QUERYABLE

OWNER	BY WHEN	COST	PREVENTS
Commune of Agadez (état civil), with the DRSP	baseline from November 2026; cause of death from March 2027	about XOF 1.5 million–2,935,700 (US$2,555–5,000), an unsourced estimate	a heat season that nobody can see, and spending nobody can judge

Heat deaths here are recorded as stroke, kidney failure, or child deaths, if at all. Gabriel Touré hospital in Bamako recorded 102 deaths on 1–4 April 2024, against 130 in all of April 2023; Niger recorded no excess, because nobody was counting. In Aden, a team counting burials from satellite images estimated about 2,100 excess burials in 2020 (Koum Besson, Checchi et al., BMJ Global Health 2021). Agadez is suited to it: burial is the same day, and the town has few cemeteries.

Corrected in verification: the cause of the Bamako deaths was never released, so they must not be called heat deaths. A burial count cannot separate heat from other causes; verbal autopsy adds that from March.

6. Food: hold a grain buffer against a convoy or fuel break, and give food trucks priority for diesel and convoy places

PARTIALLY CONFIRMED | FOOD · ECONOMIST | ADAPTATION INVENTORY: NOT QUERYABLE

OWNER	BY WHEN	COST	PREVENTS
Governor of Agadez, with OPVN (the national cereal office) and the DNPGCA	buy November 2026 – January 2027; sales April–August 2027 on the price trigger	about XOF 140 million (US$238,000) for 500 t of millet at about XOF 280/kg, our estimate; a State line, not philanthropic	empty markets in a town 900 km up a road that runs on diesel and escorts

* Suggested formulation: check with a health worker before sharing.

Agadez is a deficit market: its grain is trucked in, and its prices sit above the national average. 500 t is roughly one week of the commune's cereal use, so it is a buffer against a convoy or fuel break, not a price lever. Moderate-price sales follow from April to August (the lean-season peak is June–August, and millet rose up to 25% in May–July, per FAO GIEWS), triggered when SIMA's Agadez-commune millet price passes XOF 340/kg, about 15% above June's 295 (our threshold). Food baskets are the right tool here: in a Zinder trial they beat cash on consumption, and under 5% were resold, though they cost 15% more to deliver (Hoddinott, Sandström, and Upton 2018). The national fuel deficit from January to August 2026 was 52.6 million litres of diesel and 196.6 million of petrol.

Corrected in verification: the 12 October fuel export ban was a Commerce-minister arrêté, reportedly withdrawn on 17 October, and Niamey's supply was normalising by 23 October; no dated 2026 source on Agadez fuel was found. The SIMA bulletin lags by about a week, but the July–September 2026 bulletins are missing from the public mirror, so the trigger owner should get them direct from SIMA. The tonnage and the threshold are ours, not in any plan; no OPVN moderate-price sale in 2025–26 was found.

7. Severe acute malnutrition: keep the outpatient treatment pipeline full into 2027, with screening at the CSIs in February

PARTIALLY CONFIRMED | PHILANTHROPIST · PUBLIC HEALTH | ADAPTATION INVENTORY: NOT QUERYABLE

OWNER	BY WHEN	COST	PREVENTS
DRSP Agadez, with UNICEF	earmark before February 2027; MUAC screening in February	XOF 16,146,350 (US$27,500) for 500 courses, before in-country transport	a break in therapeutic food after January 2027

Niger's therapeutic food (RUTF) is available until January 2027, so there is no break in 2026; the money covers the 2027 gap. 500 courses at about XOF 32,300 (US$55) each, including freight (UNICEF, Q2 2026). Earmark it for outpatient care at CSIs and CRENAS, not the inpatient CRENI: in Bangladesh, community delivery cost XOF 15,300 (US$26) per DALY against XOF 789,000 (US$1,344) for inpatient care (Puett et al. 2013).

* Suggested formulation: check with a health worker before sharing.

Corrected in verification: Agadez is not Niger's nutrition hotspot. SMART 2025 found acute malnutrition of 9.0% [6.7–11.8] and severe 0.8% in Agadez region, the lowest severe rate in Niger (national 11.1% and 2.8%). The 500-child caseload is a reasonable estimate, not a count.

8. Use the dry season to inspect the Telwa dam and the flood banks by February, and repair before July 2027

PARTIALLY CONFIRMED | HISTORIAN · FRONTLINE · EMERGENCY MANAGER | ADAPTATION INVENTORY: NOT QUERYABLE

OWNER	BY WHEN	COST	PREVENTS
Commune of Agadez, with the regional hydraulics directorate	**inspect by 28 February 2027; repair before July 2027**	**not costed; unfunded**	**a repeat of the 2009 breach in the next rainy season**

On 1 September 2009 the dam on the Telwa, about 7 km from town, breached after exceptional rain. At least 3 people died (residents said dozens), and about 3,000 houses were destroyed (USAID); the government counted more than 13,000 partly or wholly destroyed. ECHO approved about XOF 587 million (US$1 million) and UNDP XOF 117 million (US$200,000) for reconstruction. The old town flooded again in September 2024 (about 100 houses and walls collapsed, the Grand Mosque damaged, 10 dead in the region) and in July 2025 (2 dead). The commune should ask the World Bank PIDUREM project (P175857) whether repairs are in scope; it currently funds only an Agadez drainage master-plan study, about XOF 57.5 million (US$98,000).

Unverified: no evidence was found that the dam has been rebuilt or inspected since 2009. This protects the 2027 rains more than this winter.

* Suggested formulation: check with a health worker before sharing.

WHO DOES WHAT

RACI CHART

R = Responsible (does the work) · **A** = Accountable (one per action; signs off and answers for it) · **C** = Consulted (asked before) · **I** = Informed (told after). Built from the owners named in the verified actions. No one here has been contacted or has agreed to a role; every row is a proposal.

Action	DRSP and CHR	WHO and UNICEF	Governor of Agadez	Protection Civile	Migrant-aid partners (IOM, Alarme Phone Sahara, MdM/MSF)	DMN	Commune of Agadez	Chiefs, imams, sultanate, and radio	Livestock Directorate and herder associations	CRN Agadez	OPVN and DNPGCA	Regional hydraulics directorate
1. Meningitis stock, catch-up, ICG file	**A/R**	**R**	**I**		**C**			**I**				
2. Desert alert number and standby vehicle	**C**		**A**	**R**	**R**		**I**	**I**				
3. Heat plan without grid power	**A/R**	**C**	**I**		**I**	**R**	**C**	**R**				
4. Herders: destock, feed, lists			**C**					**R**	**A/R**	**R**	**C**	
5. Count the dead	**R**		**I**				**A/R**	**C**				
6. Grain buffer and diesel priority			**A**				**I**	**I**			**R**	
7. Malnutrition pipeline	**A/R**	**R**						**C**				
8. Telwa dam and flood banks			**C**	**C**			**A**					**R**

What the chart shows: the DRSP is accountable for three of the eight actions (meningitis, heat, and malnutrition), the life-saving core. The governor is accountable for two (the desert protocol and the grain buffer) and informed on most others. Actions 2, 4, and 6 turn on decisions (an agreed number, a beneficiary list, a fuel priority) more than on money.

WHERE MONEY DOES THE MOST

FUNDING

The Philanthropist lens, reconciled against the ranked actions and verified. Placements are ranked by marginal lives saved per dollar. The envelope is bounded by what this place can spend before the peak, not by what it needs.

FLOOR

XOF 22,017,750 (US$37,500)

TARGET

XOF 51,374,750 (US$87,500)

CEILING

XOF 168,802,750 (US$287,500)

LARGEST UNCERTAINTY

caseloads (no 2025–26 Agadez meningitis count) and which NGOs are still authorised

Floor = placements 1 + 2: XOF 5,871,400 + XOF 16,146,350 = XOF 22,017,750 (US$37,500). Target = floor + placements 3–5: + XOF 8,807,100 + XOF 17,614,200 + XOF 2,935,700 = XOF 51,374,750 (US$87,500). Ceiling = target + placement 6, XOF 117,428,000 = XOF 168,802,750 (US$287,500). Above the ceiling the limit is who is allowed to spend: the ICRC left Niger (closure announced 5 June 2025), and registered NGOs fell from 4,700 to 1,800 (interior minister, 22 April 2026), with no public list of which were suspended. The target moved +13% in verification (from XOF 45,503,350, US$77,500), mainly because the RUTF unit price rose to about XOF 32,300 (US$55) and death counting

* Suggested formulation: check with a health worker before sharing.

was added. Already funded, not to duplicate: the 2025 humanitarian plan was about 18.5% funded (no 2026 figure confirmed), and the January 2026 CERF flood allocation (XOF 2.94 billion, US$5 million) went to Dosso and Tillabéri only. State lines, not philanthropic: the OPVN grain buffer (action 6) and the Plan de Soutien. Unfunded: the dam inspection and repair (action 8) and the desert vehicle beyond its diesel (action 2). Advocacy, not money for this window: Agadez Region is owed XOF 5.50 billion (US$9.4 million) in mining-royalty arrears for 2014–2018 (ROTAB/PCQVP study, 2019, hosted by EITI); no payment was found in 2025–26.

RECIPIENT	USE	AMOUNT	MECHANISM	BASIS	VERDICT
DRSP Agadez and the CHR	Action 1: meningitis case-management stock	XOF 5,871,400 (US$10,000)	in kind via WHO/UNICEF	Treatment XOF 232,500 (US$396) per death averted vs XOF 3.5 million (US$6,000) for late vaccination (Veeken 1998, Katsina; use as a ratio)	Partially confirmed
UNICEF pipeline, earmarked for CRENAS and CSIs	Action 7: RUTF, 500 courses	XOF 16,146,350 (US$27,500)	earmarked contribution	About XOF 32,300 (US$55) per carton including freight (UNICEF Q2 2026); XOF 15,300 (US$26) per DALY for community delivery (Puett 2013)	Partially confirmed
DRSP and the CHR	Action 3: heat alert, ORS, backed-up triage room	up to XOF 8,807,100 (US$15,000)	grant against a line-item budget	No Sahel precedent; Ahmedabad does not transfer	Unverified (upper figure)
Médecins du Monde (or MSF)	Action 2: care and basic relief for migrants and desert travellers	XOF 17,614,200 (US$30,000)	tranches; proof of current Interior Ministry authorisation first	About 11,839 migrants served in 2024; 2026 activity unverified	Partially confirmed
Commune état civil and DRSP	Action 5: death counting, November–May	up to XOF 2,935,700 (US$5,000)	small grant	Unsourced estimate (recorders, registers, phone credit)	Partially confirmed

* Suggested formulation: check with a health worker before sharing.

RECIPIENT	USE	AMOUNT	MECHANISM	BASIS	VERDICT
Croix-Rouge Nigérienne, Agadez	Action 4: pastoral cash, 1,000 households × XOF 117,428	XOF 117,428,000 (US$200,000)	cash transfer, released on the November Cadre Harmonisé and forage results	Transfer size assumed; CRN reached about 6,000 of 22,000 targeted households in 2022	Partially confirmed

The funder's one thing: the cheapest life here is about XOF 235,000 (US$400) of antibiotics already sitting at the CSI in January, not a vaccination campaign in April. The binding constraint is not money; it is which organisations are still allowed to spend it. Fund the DRSP and the groups that survived the 2025 suspensions before anything new.

Do not do this

CONTRAINDICATED · ADVICE THAT WOULD COST LIVES HERE

Advice that circulates for this hazard and would cost lives here. The second tag on each card is how well the warning itself was verified.

Telling herders that drought-protocol cash is coming

CONTRAINDICATED FOUND IN VERIFICATION (A DRAFT RESIDENT STEP)

The Red Cross drought protocol's November cash reaches 500 households in all of Niger, names no region, and its beneficiaries are pre-targeted by local community structures. A resident asking about the forage figure does not move it. A herder who waits for the money instead of selling loses the animals and the price.

* Suggested formulation: check with a health worker before sharing.

Instead: ask the CRN and your chief whether a list is being drawn up, and sell weak animals anyway (action 4).

Making shelter or aid for expelled migrants conditional on accepting "voluntary return", or handing the list of migrant compounds to the police

CONTRAINDICATED CONFIRMED

IOM admits people who want assisted return to its Assamaka centre, and in 2023 fewer than 15% of arrivals there reached shelter (MSF, Alarme Phone Sahara). People held for months at the Agadez and Arlit centres protested in 2022. Migrants in transit live in smuggler-run compounds ("ghettos") out of sight; if their location goes to the police, the doors close to health workers too (frontline lens, not separately verified).

Instead: unconditional shelter, water, and care, and compound lists kept with Alarme Phone Sahara and the health post only (action 2).

Waiting for the epidemic threshold before preparing a meningitis response

CONTRAINDICATED CONFIRMED

Reactive vaccination in Niger is often too late: a study of the 2015 serogroup C epidemics found reactive strategies had little impact (Trop Med Int Health 2017), and in a simulation on Niger data, acting two weeks earlier doubled efficiency (Maïnassara et al. 2015).

Instead: treatment stock at every CSI by 15 December and the ICG file ready to send (action 1).

Waiting for the lean season to sell weak animals

CONTRAINDICATED CONFIRMED IN DIRECTION

In 2010 herders who waited sold into a falling livestock market while grain prices rose. In June 2026 a goat still bought about one unit of millet (terms of trade 0.99); that ratio erodes through the dry season.

* Suggested formulation: check with a health worker before sharing.

Instead: sell in November–January and buy grain with the proceeds.

Handing out fans, or advising fans, for older or sick people in Agadez's dry heat*

CONTRAINDICATED PARTIALLY CONFIRMED

Meade, Notley, Kirby, and Kenny (Lancet Planetary Health, 2024) advise against fans above 35°C, especially for people over 65. Morris et al. (2021) modelled ceilings of 39°C for young adults, 38°C for older adults, and 37°C for older adults on anticholinergic drugs. In a JAMA trial in older adults, fans did not lower core temperature in extreme heat. Agadez in April–May runs at about 40–43°C with humidity below 20%. So: no fans above about 35°C for over-65s; the modelled ceiling is 37–38°C.

Instead: wet skin and clothing, shade, and water (action 3).

Building the heat plan around air conditioning

CONTRAINDICATED PARTIALLY CONFIRMED

Agadez runs on the SONICHAR coal plant and is not on the import line from Nigeria; no 2025–26 outage data for the town was found, so the case rests on cost and scale, not on documented cuts. In the April 2024 Sahel heatwave, power cuts made it hard for those who had cooling to use it.

Instead: shaded rooms, and any cooled room with solar or diesel backup (action 3).*

Sending heat or health alerts only by SMS in French

CONTRAINDICATED UNVERIFIED AS A DOCUMENTED FAILURE HERE

Most residents speak Hausa or Tamasheq, and many people at most risk, migrants and older people, are not reached by French text messages. Niger's flood alerts already go out on radio in local languages (WFP).

Instead: radio in Hausa and Tamasheq, the imams, and the chefs de quartier (action 3).

* Suggested formulation: check with a health worker before sharing.

YEAR	EVENT	TOLL	WHAT CHANGED AFTER
1969–84	Long Sahel drought: Agadez lost the equivalent of 6.8 normal years of rain; herds collapsed in 1973 and 1984, each the second dry year	No death count found; thousands of nomad families settled on Agadez's edge	The town's sedentary fringe grew from it
2009	Meningitis season	189 cases and 16 deaths in the region; 99 cases and 4 deaths in town (secondary source)	Not recorded
1 Sep 2009 (El Niño)	Telwa dam breach after exceptional rain	At least 3 dead (residents said dozens); about 3,000 houses destroyed (USAID), 13,000+ partly or wholly (government)	Dam repair named first; ECHO and UNDP money approved; no rebuild found
2009–10 (El Niño)	Second poor pasture year and food crisis	2.7 million Nigeriens severely food insecure by March 2010; acute malnutrition above 17% in parts of Agadez in late 2010	Crisis recognised earlier than in 2005; destocking still late
2015	Serogroup C meningitis epidemic, Niger	8,500 cases and 573 deaths nationally	Reactive vaccination, judged to have had little impact
2024 (El Niño)	Meningitis; April Sahel heatwave reaching 48°C in the region	Agadez attack rate 11.5 per 100,000, second in Niger; heat deaths unrecorded in Niger	Five-district vaccination response; Men5CV introduced
Sep 2024	Old-town floods	About 100 houses and walls collapsed, Grand Mosque damaged; 10 dead in the region	Nothing found
Jul 2025	Floods	2 dead	Nothing found
2025	Record expulsions from Algeria; nationwide preventive Men5CV campaign (Nov–Dec)	34,236 people expelled; 7 expulsion-linked deaths (Alarme Phone Sahara)	12.3 million vaccinated, ages 1–19

* Suggested formulation: check with a health worker before sharing.

YEAR	EVENT	TOLL	WHAT CHANGED AFTER
4 Jun 2026	Truck breakdown west of Assamaka	49 dead of thirst; 60 rescued from a second truck	No protocol found
2026	Diphtheria; a "very marked" pastoral deficit	242 of 949 national diphtheria cases to week 38 (secondary)	Not yet

The recurrence nobody has noticed: here the deaths from El Niño come the year after. 1973, 1984, 2005, and 2010 were each the second, lean-season year of a crisis. The 2009 El Niño gave Agadez a flood that autumn and its worst malnutrition the next year. The bill for 2026's failed pasture comes due between May and August 2027, after the El Niño peak and after the tracker window closes.

WHERE THE EXPERTS DISAGREE

UNRESOLVED

- **Vaccinate preventively by health area in December–January** (Skeptic) versus **the campaign has already happened** (verification: Men5CV for ages 1–19, nationwide, November–December 2025). The ranking keeps catch-up doses only; a district coverage check would settle whether Agadez needs more.
- **Blankets for expelled people save lives** (Emergency manager) versus **no cold death among them is documented** (verification). Kept as basic relief, not ranked as a life-saver.
- **Buy grain to hold prices down** (Food) versus **500 t is about one week's use** (verification). Kept as a buffer against a convoy break.
- **Collect the mining-royalty arrears as risk finance** (Economist) versus **unpaid for more than seven years, so not money for this window** (verification). Kept as advocacy.
- **The town has more than 300,000 people** (the former mayor, cited by the Comboni missionaries, 2026) versus **official projections far lower**. A census would settle it; until then no town share of the regional deaths is given.

WHAT NOBODY COVERED

BLIND SPOT

No lens interviewed anyone in Agadez. Nothing was found on gold-mining workers returning from Djado or Tchibarakaten in 2025–26, although the June 2026 dead were gold-site workers. No Agadez power-outage data, no current status for Sahara FM, and no verified quarter names were found. The 2027 lean season (May–August) lies outside the tracker window and is likely the larger mortality risk.

* Suggested formulation: check with a health worker before sharing.

HOW TO READ THIS METHOD

The panel is author-built. Eight simulated expert lenses were used to generate these recommendations. Where they agree, that is a strong hypothesis, not a consensus of the field. Every recommendation was then independently checked for evidence, transferability to this location, and feasibility before the peak; the verdict tags carry the result.

Mortality figures are published at region scale, for Agadez Region. They are not figures for the town itself. The El Niño projection covers *heat only*; drought, flood, fire, smoke, and crop failure are additional to it and are carried here by the historical record rather than by a published number.

HOW THIS BRIEFING WAS PRODUCED METHODOLOGY

- **Scope.** The location was taken as given: Agadez town (commune urbaine d'Agadez), the capital of Agadez Region, Niger. Mortality figures exist only at region level; town detail is used for hazard exposure and action design, not for death counts. The town's population is contested, so no town share of the regional figure is given.
- **Exposure.** Climate Impact Lab El Niño Heat Mortality Tracker data, file `v261026_adm1_output_BASE_rounded_cust.csv` (version of 26 October 2026, pre-peer-review): Agadez Region (GADM NER.1_1), 310 additional heat deaths June–September 2026, 93 forecast October 2026 – March 2027 (October 24, November 20, December 0, January 3, February 4, March 43), 410 in total; Niger, 10,600; the world, 857,400. This update supersedes the September report *451,000 at Risk*. Method: ECMWF SEAS5 seasonal forecasts applied to temperature–mortality relationships against a 1996–2025 baseline. The figures count heat only; we inferred this from the data, because cold countries show exactly zero in every winter month. They are never added to the Impact Map's long-run projections.
- **Expert panel.** Eight simulated expert lenses (emergency manager, public health scientist, frontline responder, skeptic, economist, historian, philanthropist, and food supply chain analyst) each researched this place independently and in parallel, from web sources in French and English, to a fixed brief.
- **Reconciliation.** Duplicate actions were merged and backing lenses recorded. Convergence was tested for shared bias (whether the evidence behind an agreed action came from a setting like this one) before it counted in an action's favour. Contradictions were kept, not resolved by dropping a side. Actions were ranked by expected lives saved per unit of effort before the peak, each was given a named payer and trigger or marked unfunded, and the funding placements were reconciled against the ranked list. The mining-royalty arrears were dropped as an action and kept as an advocacy ask.

* Suggested formulation: check with a health worker before sharing.

- **Verification.** Five verifier agents (meningitis; financial instruments; migrants and heat; food and nutrition; the funding envelope, death counting, and the dam) checked every recommendation on three tests: evidence (the primary source and its effect size), transferability (whether that evidence came from somewhere like here), and feasibility (whether it can be done here before the peak). Financial actions were also checked for availability in this jurisdiction, a named payer, a trigger threshold, and whether they already exist; food actions for published triggers and timing. The funding envelope was checked for real recipients, published unit costs, and duplication, then re-summed. The 9 For you steps were verified on the same tests, judged for one resident.
- **Who does what.** The RACI chart is built from the owners named in the verified actions, with one accountable party per action. It is a proposal: no organisation named in it has been contacted or has agreed to a role.
- **Verdicts.** *Confirmed*: evidence, transferability, and feasibility all hold. *Partially confirmed*: holds after the corrections shown. *Unverified*: could not be checked to a primary source. *Contraindicated*: would do harm or waste the window here.
- **Adaptation Inventory.** Not usable in this run. The inventory renders in the browser and returned no entries to the agents' fetcher, so no precedent could be confirmed there. No action here should be read as having no precedent; none was checked.
- **Money.** Costs are shown in West African CFA francs (XOF) with US dollars in parentheses, at XOF 587.14 = US$1. The rate comes from the fixed peg of XOF 655.957 = €1 and the ECB reference rate of €1 = US$1.1172 (30 October 2026). Figures published in Swiss francs (Red Cross protocols) are converted at about XOF 700 per CHF, an approximation via the euro peg.
- **Reader.** No role was given, so the For you section is written for a resident. Its steps were taken from the verified actions and from what the panel found about households here. Any step not drawn from a ranked action was verified on the same three tests; two (the brazier and the water store) could not be, and are labelled as general safety advice. The rest of the report is written for the region and its partners.
- **Language.** A French version accompanies this one, because French is the language of Niger's administration. Most residents of Agadez speak Tamasheq or Hausa. A Hausa edition was added on 31 October 2026; no Tamasheq version was made, and action 3 asks for the key messages to go out on radio in both languages.
- **Health wording.** Passages where a wording error could hurt someone (meningitis care, braziers and carbon monoxide, cooling and fans, the home ORS recipe, desert travel, and when to seek care) are marked "* Suggested formulation: check with a health worker before sharing." They were checked against published sources, not by a health professional. They are suggestions, not medical advice, and anyone passing them on should confirm them with a doctor, pharmacist, nurse, or community health worker first. The marker was added on 2 November 2026.

* Suggested formulation: check with a health worker before sharing.

- **Rules.** No cost is derived by dividing a published total, and no local figure is invented; where data is thin, the report says so and shows the containing geography's number with its arithmetic. Our own estimates (the grain tonnage and price threshold, the pastoral transfer size, the death-counting and vehicle costs) are labelled as such. Every source is listed below with its verification status.
- **Limits.** The panel and verifiers are AI research agents working from web-available sources, not human experts, and no one in this place was interviewed. The national statistics office's Agadez demographic PDF returned HTTP 500, and its quarterly bulletin would not download. The FEWS NET October 2026 outlook and the October–November 2026 Cadre Harmonisé were not yet published. SIMA's July–September 2026 bulletins were not on the public mirror. No coordinates from a source were found for the CHR, Assamaka, or the Telwa dam, so no action is pinned on the map; all are placed by area. This was the first of two briefings in this session.

References · status after verification

Climate Impact Lab, El Niño Heat Mortality Tracker data v261026 (downloaded 30 Oct 2026); tracker · confirmed

Meningitis and diphtheria: WHO WER 101(38), Men5CV preventive campaign; government statement, nationwide campaign; WHO AFRO, 2024 epidemic; Veeken et al. 1998; Maïnassara et al. 2015; reactive strategies, 2015 epidemics; Gavi, Men5CV; INAM, free care; free-care arrears · partially confirmed. mesvaccins.net, diphtheria 2026; 2009 meningitis; WHO DON, 2015 · secondary or not re-checked

Desert and migrants: 4 June 2026 deaths; survivors' account; Alarme Phone Sahara, 2025 expulsions; IOM budget cuts; Médecins du Monde, Agadez; Assamaka alert, 2023; IOM admission and return; MSF, Assamaka shelter; 2022 protests; Agadez ghettos · confirmed or partially confirmed

Heat, power, and radio: Hess et al. 2018; hypertension, Niamey; ACASIS; ECMWF SEWA; Meade et al. 2024, fans; Morris et al. 2021, fans; JAMA, fans in older adults; WWA, April 2024 Sahel heat; Agadez grid; hybrid plant; Sahara FM arrests; WFP, local-language flood alerts; home ORS recipe; MSF, ORS · partially confirmed. brazier deaths, Buea; Santé publique France, CO · unverified for Niger

Herders and money: CILSS pastoral note, Sep 2026; FEWS NET Niger; IFRC EAP2024NE04; CERF drought framework; CERF flood activation, Jan 2026; WFP/ARC reinsurance; Plan de Soutien 2022; destocking lessons, 2010; LEGS case study; CRN 2022 response; mining-royalty arrears study; ICRC exit; NGO suspensions; Niger humanitarian plan; ECB reference rate · partially confirmed (CERF for Agadez and ARC: unverified)

Food and nutrition: SIMA bulletin n°349, June 2026; FAO GIEWS Niger; Hoddinott et al., Zinder trial; fuel deficit and export ban; ban withdrawn; Niamey supply resuming; UNICEF RUTF, Q2 2026; SMART 2025 Niger; Puett et al. 2013 · partially confirmed

Death counting: Bamako, April 2024; Aden burial counts; West Africa heat toll, 2024 · confirmed (heat cause in Bamako not established)

Floods and history: 2009 dam breach; 2009 reconstruction money; Sept 2024 old-town floods; 2024 floods, Grand Mosque; July 2025 floods; PIDUREM drainage study; 1969–84 rainfall deficit; herd collapses; USAID, 2009–10; FAO, 2010; population claim (Comboni, 2026) · partially confirmed (dam rebuild: unverified)

Exposure data · Climate Impact Lab, 451,000 at Risk · Impact Map · Adaptation Inventory

Generated 30 October 2026 for Agadez, Agadez Region. El Niño Ready is volunteer-run and is not affiliated with Climate Week NYC, The Climate Group, or Columbia University.

* Suggested formulation: check with a health worker before sharing.